
FINDINGS FOR THE BOARD

Organisational Audit

Haven Paediatrics

What we found when we looked underneath a hospital that is, on every measure your families can see, doing the hard part well.

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Four independent sources, and they agree with each other

Your people, your families and your own records were tested separately. Where they disagree, this report says so.

DEPARTMENTS INTERVIEWED

9

One to one, on site, 4 August

STAFF RESPONSES

23

Anonymous, safety and culture

CAREGIVER RESPONSES

16

Anonymous, patient experience

DOCUMENTS REVIEWED

34

January to July, recomputed from source

Source	What it covered
Field validation	Nursing, front desk, billing, pharmacy, laboratory, administration, business development, doctors, patients
Staff survey	Nine sections adapted from a recognised hospital safety-culture instrument, plus items on readiness, reward and ownership
Caregiver survey	Access, communication, safety, environment and cost
Documentary review	Sales files, admission record, pharmacy report, claims file, tariff, fee schedule, board decks, half-year report, weekly report, six procedures, staff list, four rosters

**Every figure taken from your records was recomputed from the underlying patient rows, not read off a total line.
Where data is missing we say it is missing.**



The care is ahead of the systems holding it up

ORGANISATIONAL MATURITY

2.5 / 5

Developing. Opened at 3.0

PATIENT EXPERIENCE

4.60 / 5

16 caregiver responses

STAFF SAFETY GRADE

22 / 23

rated very good or excellent

STAFF UNDER ONE YEAR

12 / 16

none beyond two years

Your patients rate you highly and your staff rate the safety of the care they give as good. What sits underneath is not yet a hospital's infrastructure. Processes are written but untaught and unmeasured. The revenue record is a nursing handover sheet. Four registered nurses covered the half year. No pharmacist is rostered on any day.

Haven is not underperforming. It is a hospital whose performance rests on particular people rather than on systems that would survive their leaving.



Five things are working and should survive any change

- **Patient experience.** Caregivers rate you 4.60 of 5. 13 of 15 would definitely recommend you. Access, cleanliness and confidence in the medical team are the strongest items.
- **Clinical delivery.** Pathways are coherent and consistently described by every department. Emergency equipment readiness is trusted. Staff report errors openly.
- **Pharmacy discipline.** Monthly counts, consumption-led purchasing, expiry lists circulated months ahead, weekly procurement. The internal benchmark for every other department.
- **The billing front end.** Bills assemble from the clinical record without manual work. Eligibility is checked before treatment. Errors are rare and a documented overpayment was refunded within a week.
- **Teamwork.** Mutual respect 4.35, pulling together under load 4.22, handover clarity 4.23. The hard part of culture is already there.

Every recommendation in this report is designed to protect these five while fixing what sits under them.



Three questions only the board can close

These are not items for a workplan. Until they are answered the rest of the plan cannot be sequenced honestly.

01

Regulatory currency

Registration is confirmed and this year's renewal is pending. What remains is that your monthly reports said the facility was still a clinic and unregistered, every month January to April, in a period when it held registration and was admitting overnight, transfusing and running neonatal care. Both lines then disappeared without correction.

02

The establishment

Four registered nurses covered the first half of the year. Eight days in July had two people covering the ward, emergency room, neonatal unit and triage for a full 24 hours. On four of those days one of the two was not a nurse. No pharmacist appears on any day of the roster.

03

The record

Your nursing procedure has nurses filling in the revenue spreadsheets at the end of shift, so those sheets sit beside your clinical system rather than being the financial record. June HMO revenue is absent from the sales record, though the claims were submitted and paid. The system underneath looks sound.

The first is now largely closed. The renewal is the outstanding item, and the four months of inaccurate reporting behind it is the finding that carries into Section 10.



Registration is held. The reporting said otherwise for four months.

Item	Status	Action
Facility registration	Obtained, confirmed by management	Closed. Hold the certificate on the governance file
Current year renewal	Pending	Submit and evidence. Do not let it run into another quarter
Health insurance scheme registration	Not separately confirmed	Confirm in writing. The monthly reports are not a reliable guide either way

The substantive question is answered. What it leaves is a reporting problem, and it is the clearest single illustration of the finding in Section 10.

What the monthly reporting recorded	What was actually the case
Still running as a clinic, every month January to April	Registration held, and twenty-four hour admissions running since August 2025
Registration not yet obtained, same period	Registration had been obtained
Both statements removed from May with no correction recorded	A correction would normally be minuted. None was

Two functions of the same organisation reported opposite positions on the same fact, in the same months, to the same board. One was right. Nobody reconciled them.



The hospital is staffed like the clinic it used to be

Function	Established	What the roster shows
Registered nurses	4 through the half year	The fifth and sixth joined 14 and 15 July. Your half-year report describes six covering January to June
Medical officers	3	All 31 days of July on a three-day rotation. No rest day, no leave cover, no relief named
Pharmacist	1, part time	Appears on no day of the July pharmacy roster. Two technicians verify paediatric weight-based doses
Laboratory scientists	2	One scientist covers 72 consecutive hours in three-day blocks. Before 16 April, one in total
Consultant rota	In place	Not supplied for review, so consultant cover could not be tested against the medical officer rota
HMO officer	0	The post that owns every gate in the insured revenue cycle, including the daily claims audit, does not exist

Eight days in July were covered by two people across a full 24 hours. On four of those the cover was one registered nurse and a pharmacy technician, who on three of them was also rostered to the pharmacy.

Every monthly report states a one to two patient to nurse ratio, unchanged across six months in which monthly encounters rose from 174 to 311. A ratio that does not move when the numerator triples is not a measurement.



The spreadsheets sit beside the system, not above it

Your nursing procedure instructs nurses to bill opened medications from memory at the end of a shift, and to fill in the revenue spreadsheets before handover, with the obligation passing to the next shift if they have not managed it. Every finding below follows from that one instruction.

What is missing or wrong	Amount	How it is known
June HMO revenue absent from the sales record. The claims were submitted and paid	N7,510,645	What is missing is the sales record of it. Corroborated by N2,097,155 of June pharmacy dispensing with no matching entry
April private pharmacy revenue. Column total never written	N1,394,020	Patient rows sum 14.6 percent above the month total row
May insured revenue across three columns	N225,893	Total rows are hardcoded values, so rows added later are silently excluded
Service revenue on the largest episode of the period	N4,000,000	Appears once in a weekly report and is excluded from that report's own total
Pharmacy stock fails to roll forward in every month tested	N562,272	The physical count is written into the ledger instead of reconciled against it

1,952 of the 2,413 lines in the tariff file we were given carry a price of zero. Either that is not the list your team prices from, and we should see the one that is, or pricing is being set at the point of billing. Both need answers and they are different answers.



Your board pack does not reconcile to your transaction record

Month	Reported to board	Transaction record	Variance
January	N12,698,284	N4,696,654	+N8,001,630
March	N8,543,518	N16,410,449	(N7,866,931)
June	N14,638,870	N14,638,870	Nil
Half-year total	N68,159,516	N68,063,877	+N95,639

The half-year total is accurate to 0.14 percent. Every individual month before June is wrong. January is overstated by N8.00M, March understated by N7.87M, and those are the same money. June reconciles to the naira, which shows the method works when the month is prepared properly.

- N6,395,450 of neonatal revenue reported in January, a month whose entire private revenue across all lines was N4,696,654.
- Insured revenue reported above what was actually claimed by N386,887, with the February and May variances each tracing to a single payer to the naira.
- Laboratory revenue reported as an identical figure in March and April, matching neither month, carried through two board cycles.

There is an innocent explanation available and it fits most of this. We recommend a reconciliation done outside the operations function, and nothing beyond that. The board should not reach a conclusion before that work is done.



Caregivers rate you 4.60 out of 5

OVERALL

4.60 / 5

mean across all ten items

WOULD DEFINITELY RECOMMEND

13 / 15

to another family

RESPONSES

16

anonymous, 4 to 14 August

Strongest	Softest
Easy to book and be seen 4.88	Charges and payments explained clearly 4.00
Clean and comfortable 4.88	Waiting time 4.31
Treated with respect and kindness 4.81	Knowing what was happening with the child 4.33
Confidence in the medical team 4.81	Care explained understandably 4.56

All three soft edges are about information, not clinical care. They are the cheapest things in this report to fix. The comments ask for remote consultation, results by email, and clearer medicine costs under insurance.



Safety rates well. The deal does not.

Strongest		Softest	
Would recommend Haven as a place to receive care	4.48	Recent resuscitation training	2.80
Staff treat each other with respect	4.35	Pay, commission and rewards are fair and clear	2.94
Clear what I am responsible for	4.35	Rewards encourage careful work, not just volume	3.35
Emergency resuscitation readiness	4.45	Free to question those with more authority	3.33
Information shared clearly across shifts	4.23	Blamed rather than helped after reporting (reverse)	2.41

- **Tenure.** 12 of 16 who disclosed it have been here under a year. None beyond two.
- **The two asks, repeated across every department.** More hands for patient safety, and welfare: health insurance, pension, fair and timely pay, recognition, clinical training.
- **Tone.** Almost every critical comment is prefaced with warmth about the place. These are people asking to be invested in by an employer they like.



Processes exist. They are not followed. That has four causes.

01

Nobody was taught them

Procedures are emailed and staff are expected to absorb them. Two of three nurses interviewed had never had any training on them. No induction, no sign-off, no refresher.

02

Nobody measures them

No compliance audit, no spot check, no exception report. Supervision is real but informal, and triggered by error. Adherence is invisible until it fails.

03

The workforce is new

12 of 16 under a year, none beyond two. A workforce this new cannot inherit institutional habit. There is nobody to inherit it from.

04

Several cannot be followed

The daily claims audit is assigned to a post nobody holds. The witness control needs two officers where one is rostered. The nursing procedure prohibits what it has just instructed.

Writing more procedure will not close this, and enforcing what exists would in several cases enforce something impossible. Rebuild the core processes with the people who run them, then train, sign off and audit, in that order.



Rated across the employee lifecycle: 1.4 out of 5

Stage	Rating	What is missing
Workforce planning	1	An establishment plan by role and shift, linked to volume and acuity
Selection	2	Role profiles, structured interview, competency assessment, credential verification
Onboarding	1	Defined induction, procedure walkthrough and sign-off, 30 and 90 day checkpoints
Training	1	Annual calendar, mandatory clinical currency, competency framework, training record
Performance	1	Objectives set by management, monthly one to one, quarterly review. Staff were asked to write their own indicators
Reward	2	Published grade and pay structure, transparent review basis, a decided benefits position
Engagement	2	Regular all-staff forum, department briefing rhythm, closed feedback loop. Two general meetings in a year
Retention	1	Exit interviews, tenure and turnover reporting, cost of turnover model

The opening review scored this workstream 3.0 on the strength of rosters and departmental schedules. Those are scheduling artefacts. Haven has administration; it does not yet have human resources.



Two legitimate instincts at owner level, no agreed resolution

Weighted to the workforce

Right that Haven's entire quality proposition rests on people, and that the turnover exposure is real and under-measured.
Cautious about applying commercial pressure to a young team doing difficult work.

Weighted to the commercial model

Right that a young facility committing to benefits it cannot sustain damages the workforce more, not less. Cautious about fixed costs a fifteen month old facility has not yet earned.

The problem is not that both views exist. It is that the trade-off has never been settled, written down, and given to management as a rule. So the answer to any question depends on which owner is asked, and management learns to avoid asking. That is how a benefits decision goes unresolved while appearing in every staff survey.

The fix is five or six written operating principles that convert both instincts into one decision rule. Section 19 of the report drafts them. The central one: workforce investment is funded from identified yield improvement, so it is real and it is earned.



Your two highest-value assets are not managed for yield

The neonatal unit

The highest-yield service you run and the hardest to replicate. A staffed, equipped, empty cot costs almost as much as an occupied one. It is not the focus of the outreach effort, and neonatal referral is a clinical sale, not a general marketing one. Your business development team is working genuinely hard and has not been given the clinical content to hold that conversation with paediatricians, obstetricians and maternity centres.

The laboratory

Sound workflow, accuracy validated against an external reference laboratory. No economic instrumentation at all. Nobody measures how many reportable results a reagent pack produces against how many it should, so wastage from repeat testing, calibration and low batch utilisation is invisible. Under-loaded analysers make every result more expensive. Servicing is reactive, and samples are referred out at unquantified cost while machines are repaired.

Both gaps close with content and measurement, not headcount. Specialist clinics delivered 119 encounters in six months across thirteen consultant relationships. Speech therapy delivered one. Nephrology, a lead promoted specialism, delivered none.



Space is now on the critical path, and two items are compliance

Constraint	Nature	Why it matters
General ward drainage	Compliance	The ward floods when it rains. Infection prevention, electrical safety and service continuity are all implicated
Controlled medicines storage	Compliance	Correctly segregated but insufficient capacity. The exposure grows with volume
Inpatient bed spaces	Capacity	Named repeatedly by staff as the limit on patient care. Caps inpatient growth
Caregiver access	Experience	Parents repeatedly raise the staircases and ask for lift access
Consultation rooms	Capacity	Constrains sessional specialist clinics, otherwise a low capital growth option

Drainage and controlled medicines storage should be remediated now, as compliance items, not as part of any expansion case. They are inexpensive against their risk and neither should wait on a capital decision.

One of your staff put the strategic version better than we could: what we have is working, but we are growing bigger.



The average moved a little. The distribution moved a lot.

Workstream	Opening	Now	Why
Patient experience	2.0	4.5	Now evidenced, and the strongest result in the audit
Human resources	3.0	2.0	Rosters are scheduling, not people systems
Performance management	3.0	2.0	Reporting exists; nothing sits above it
Clinical operations	4.0	3.0	Delivery strong, establishment thin
Clinical governance	n/a	1.5	Separated out. No incidents, audit or compatibility method
Revenue cycle	2.0	2.0	Front end works; the record behind it does not
Finance	2.0	1.5	Now evidenced. No accounting record was produced
Risk and compliance	2.0	1.5	No register, no audit, licensing unresolved
Overall	3.0	2.5	Two workstreams rose sharply, seven fell

Everything that touches the patient scored better than we assumed. Everything that supports the patient scored worse. Haven has been building outward and has not yet built inward.



Six things that should not wait for a plan or a board meeting

- **Submit the registration renewal.** Registration itself is confirmed. Get the current year renewal in and confirm the health insurance scheme position in writing.
- **Start a fridge temperature log today.** Every day without one adds to a period of vaccination for which cold chain cannot be evidenced. N5.3M of vaccination revenue sits behind it.
- **Roster a pharmacist on every dispensing day.** Paediatric doses are weight calculated and verification currently happens without a pharmacist on the premises.
- **Set a floor of two registered nurses in the building.** And stop rostering non-nursing staff as nursing cover.
- **Commission a load calculation for the neonatal unit.** The primary generator is recorded down for six months while ventilated neonates were cared for.
- **Turn on nightly backup of the clinical record system.** It is the record for both care and billing, and none operated through the half year.

None of these six is a recommendation in the ordinary sense. They are the conditions under which the rest of this work can sensibly be implemented.



Settle the direction, then build the people system, then measure it

Window	What gets done
Days 0 to 30	Board alignment session producing written operating principles. Clinical incident register open. Departmental objectives published, five per department. Benefits position decided and communicated. The last pay round explained.
Days 30 to 60	Structured induction built and run retrospectively for everyone hired in the last twelve months. Performance cycle live. Core procedures redesigned with the teams who run them. Complaints framework. Laboratory yield audit. Neonatal referral programme.
Days 60 to 90	Adherence audit against the redesigned procedures. Retention diagnostic from payroll. Finance review and receivables. Continuous patient experience measurement. Referral tracking. Senior operations leader recruited.
Months 4 to 12	Clinical governance committee and audit programme. Competency framework. Published grade structure. Governance instrument set. Service line reporting by contribution. Estate decision executed. Membership and bundled care launched.

One sequencing rule matters above the rest. Do not audit adherence before the procedures have been redesigned and people have been inducted. Auditing people against standards they were never taught converts goodwill into resentment, and goodwill is the asset this whole plan depends on.



Five decisions, and what we would do

Decision	Options	Our recommendation
Where growth comes from	Deepen the current site, extend it, open a second site, or extend reach without consuming the building	Deepen and extend reach now. Decide the extension within two quarters on the estate review. No second site until maturity reaches 3.5
Who runs the hospital	Founder-led and strengthened, a senior operations leader, or an interim then a permanent hire	Interim operating capability now so the ninety day plan lands, then recruit permanently into a role proven rather than imagined
The workforce proposition	Lean employer, credible employer, or premium employer	Credible, phased, funded explicitly from yield. Publish the structure and explain the last round immediately; both cost nothing
The service portfolio	Continue selling episodes, or add membership and bundled care	Design the membership and bundle programme now, launch when capacity and measurement can carry it. The prize is reversing the cash cycle
How it is funded	From operations, debt against the estate, or partner capital	Operations for everything in year one. Debt only once the estate review quantifies the return and service line reporting exists

Settle the workforce proposition before committing capital to growth. It is what makes the growth deliverable.

WHERE THIS LEAVES YOU

The hard part is done. The machinery is not.

Haven is fifteen months old and already delivers care that families trust and that staff will defend anonymously. What is not built is the induction that carries the standard to the next nurse, the record that proves what was done, the rota that means the standard does not depend on who is on shift, and the measurement that tells you before a family does.

None of that requires capital you do not have or talent you cannot hire. Most of the first ninety days is decisions, documents and discipline.

Close the three questions. Then build the people system. Then grow.